

CONTROLLING OFFICER'S REPLY**HHB059****(Question Serial No. 2008)**Head: (140) Government Secretariat: Health BureauSubhead (No. & title): ()Programme: (2) HealthControlling Officer: Permanent Secretary for Health (Thomas CHAN)Director of Bureau: Secretary for HealthQuestion:

It is mentioned in the Programme that the Health Bureau will enhance the Elderly Health Care Voucher Scheme in 2023-24. In this regard, will the Government inform this Committee of:

1. the numbers of voucher claim transactions and the total amounts of vouchers claimed in the past 3 financial years, broken down by healthcare service provider;
2. the estimated expenditure on the scheme in 2023-24; and
3. any plan to increase the voucher amount? If yes, what are the details? If no, what are the reasons?

Asked by: Hon CHENG Wing-shun, Vincent (LegCo internal reference no.: 39)Reply:

1.

The tables below show the number of voucher claim transactions, the amount of vouchers claimed, and the average amount per claim transaction by types of healthcare service providers enrolled in the Elderly Health Care Voucher Scheme (EHVS) in the past 3 years:

Number of Voucher Claim Transactions

	2020	2021	2022
Medical Practitioners	1 957 092	1 917 943	1 954 032
Chinese Medicine Practitioners	1 376 436	1 542 578	1 647 630
Dentists	246 844	308 343	288 532
Occupational Therapists	4 640	7 224	4 177
Physiotherapists	39 669	48 107	37 603
Medical Laboratory Technologists	15 324	20 033	14 593
Radiographers	14 386	19 373	20 761

Nurses	6 903	11 295	9 376
Chiropractors	8 826	9 357	8 841
Optometrists	158 127	196 046	161 156
Sub-total (Hong Kong):	3 828 247	4 080 299	4 146 701
University of Hong Kong - Shenzhen Hospital (HKU-SZH)	18 962	35 953	32 356
Total :	3 847 209	4 116 252	4 179 057

Amount of Vouchers Claimed (in \$'000)

	2020	2021	2022
Medical Practitioners	947,488	1,027,990	1,059,052
Chinese Medicine Practitioners	634,851	788,617	854,324
Dentists	276,556	355,444	343,327
Occupational Therapists	5,383	7,503	4,518
Physiotherapists	15,191	19,238	17,743
Medical Laboratory Technologists	13,706	20,552	13,393
Radiographers	14,700	22,603	24,635
Nurses	8,753	11,049	9,878
Chiropractors	5,127	5,760	5,080
Optometrists	225,903	284,753	233,912
Sub-total (Hong Kong):	2,147,658	2,543,509	2,565,862
HKU-SZH	5,507	12,103	10,949
Total :	2,153,165	2,555,612	2,576,811

Average Amount of Vouchers per Claim Transaction (\$)

	2020	2021	2022
Medical Practitioners	484	536	542
Chinese Medicine Practitioners	461	511	519
Dentists	1,120	1,153	1,190
Occupational Therapists	1,160	1,039	1,082
Physiotherapists	383	400	472
Medical Laboratory Technologists	894	1,026	918
Radiographers	1,022	1,167	1,187

Nurses	1,268	978	1,054
Chiropractors	581	616	575
Optometrists	1,429	1,452	1,451
HKU-SZH	290	337	338

2. The financial provision in 2023-24 for the EHVS is \$3,769.9 million.

3.

The Government launched the EHVS in 2009. It aims at providing financial incentives for elderly persons to choose private primary healthcare services that best suit their health needs and providing them with additional healthcare choices on top of the existing public healthcare services. Currently, the EHVS subsidises eligible Hong Kong elderly persons aged 65 and over with an annual voucher amount of \$2,000 (accumulation limit of \$8,000).

In accordance with the Primary Healthcare Blueprint launched by the Government in December 2022, the Government will strive to direct resources towards primary healthcare services with an emphasis on strengthening chronic disease management and reinforcing the different levels of prevention. Experience on the use of Elderly Health Care Vouchers (EHCVs) since its introduction as a pilot scheme in 2009 indicates that the current mode of completely undesignated and unguided use of EHCVs without monitoring of the healthcare services being provided is not particularly conducive and effective to achieving the objective of enhancing primary healthcare for the elderly. As such, the Government will improve the EHVS by incentivising elders to use their vouchers for continuous preventive healthcare and chronic disease management with healthcare service providers, such as health assessment, chronic disease screening and management or other government initiatives.

As announced in the 2022 Policy Address, the Government will roll out a three-year pilot scheme to encourage the more effective use of primary healthcare services by the elderly, through increasing the annual voucher from the existing \$2,000 to \$2,500. The additional \$500 will be allotted to elderly persons' account upon claiming at least \$1,000 from the voucher for designated primary healthcare services such as disease prevention and health management. The additional amount should also be used for those designated services. The Government's view is that the direction for EHVS enhancements should be towards designated or guided use of EHCVs for appropriate healthcare services especially evidence-based and protocol-driven disease prevention or health management.

Hong Kong has one of the most rapidly ageing of population in the world and the pace of ageing will peak in the upcoming decade. The population aged 65 and over will increase from 1.5 million in 2021 to 2.52 million in 2039. Both the number of elderly persons using vouchers and the financial commitments involved will continue to increase substantially. In considering whether to further increase the voucher amount, we will have to have full regard to the effectiveness of the EHVS in achieving our health policy objectives, the situation of Hong Kong's public and private primary healthcare services, and the long-term implications on public finance. Optimising the use of resources invested in the EHVS so as to achieve our objective to enable the elderly to make good use of their vouchers on primary healthcare services for disease prevention and health management is essential for enhancing the health

of the elderly and the sustainability of the healthcare system. Continuing to increase or expand EHCVs for undesignated and unguided uses for unmonitored healthcare services, and allowing use of EHCVs for secondary/tertiary healthcare services would not be sustainable policy-wise and financially and is thus not on our policy agenda.

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